

A. Service Specifications

Mandatory headings 1 – 4: mandatory but detail for local determination and agreement
Optional headings 5-7: optional to use, detail for local determination and agreement.

Service Specification No.	
Service	Secondary Care Phlebotomy LES
Commissioner Lead	Karen Barry and Nicola Jones
Provider Lead	General Practice
Period	1 April 2026 – 31 st March 2027 (3 months' notice will be required to terminate this contract by either party)
Date of Review	31 st October 2026

1. Population Needs

1.1 Local context

This enhanced service is commissioned by NHS Devon (ICB) .

The service is for the provision of a phlebotomy service for patients requiring general blood tests that relate to hospital specialist-initiated activity, e.g., blood tests relating to outpatient appointments.

The aim of this commissioned service is to ensure accessible and convenient phlebotomy provision is available for the population of Devon, who require a blood test where initiated/requested by a secondary care provider. Recognising that this provision is not primary cares responsibility within the GP core contract or other local enhanced service. This service is not intended to replace the hospital phlebotomy service but to support it.

By providing this service in primary care it will relieve the pressure on the secondary care service and provide a much more convenient service for patients requiring blood tests for this purpose. It is recognised that the 10 year plan and associated guidance steers us towards providing services, where possible and viable, within neighbourhoods and thus closer to patients usual place of residence.

Whilst hospital specialist-initiated blood requests are the responsibility of Acute Trusts, in signing up to this service, the contractor (GP practice) is committing to providing a service to patients who request hospital specialist-initiated blood tests that fall within the below acceptance criteria.

2. Outcomes

2.1 NHS Outcomes Framework Domains & Indicators

Domain 1	<u>Preventing people from dying prematurely</u> Reducing deaths in babies and young children	X
Domain 2	<u>Enhancing quality of life for people with long-term conditions</u> <u>Health related quality of life for people with long term conditions</u> <u>Ensuring people feel supported to manage their condition</u> <u>Improving functional ability in people with long term conditions</u>	X
Domain 3	<u>Helping people to recover from episodes of ill-health or following injury</u> Emergency admissions for acute conditions that should not usually require hospital admission	X
Domain 4	<u>Ensuring people have a positive experience of care</u> Patient experience of \primary Care Patient experience of hospital care Friends and family Test <u>Improving children and young people's experience of healthcare</u> <u>Improving peoples experience of integrated care</u>	X X
Domain 5	<u>Treating and caring for people in safe environment and protecting them from avoidable harm</u> Patient safety Incidents reported Reducing the incidence of avoidable harm	X

3. Scope

3.1 Aims and objectives of service

The provider (GP Practice) will be required to provide the following phlebotomy service to patients:

- Take blood samples that relate to hospital specialist-initiated activity, e.g., blood

tests relating to outpatient appointments.

- Following a request from the patient for a hospital generated blood test; provide the blood test service at an appropriate time, location, and environment.
- Provide the equipment necessary to provide the service. This includes ensuring that the equipment meets the requirements of the local laboratory service.
- Take blood sample and label
- Record blood test on clinical system using the following SNOMED code: -
Phlebotomy generated from secondary care done by practice
XaK2I (165791000000107)

There is no requirement to:

- Record any additional information as this is a provision of phlebotomy only service.
- Operate any aspect of a recall system
- Review results or take any action associated with results

Providers are encouraged to consider collaboration if more appropriate either by Primary Care Networks or other agreed grouping e.g. neighbourhoods.

3.2 Service description/care pathway

- The provider who is treating the patient will supply the patient with a blood test request form or label that will ensure that the results will be returned to and followed up by that requesting department. The system to achieve this is not currently operating in all localities however it is possible for results of blood tests to be copied to the Consultant under whom the request was made.
- Patient advised by acute trust specialist team to arrange their blood test at their GP practice.
- Provider (GP Practice) to book appropriate time slot with appropriately skilled health care professional e.g. phlebotomist, nurse, or HCA
- Take blood sample and label
- Record blood test on clinical system using the following SNOMED code: -
Phlebotomy generated from secondary care done by practice
XaK2I (165791000000107)

3.3 Population covered

The registered GP provider population or other agreed GP provider PCN or neighbourhood population.

3.4 Any acceptance and exclusion criteria and thresholds

Acceptance

- Patient with a blood test form clearly identifying results have been requested by (state specialist and department) and will be followed up by that department.
- General blood tests that relate to hospital specialist activity, e.g., blood tests relating to outpatient appointments.
- Fasting blood test

Exclusions

- **Patients with no blood test request form, as results must be followed up by the requesting specialist. This clause will come into operation as soon as the laboratory requesting systems are fully enabled to make it operational. Ensuring this is in place will be an important component of the review of this LES.**
- Children under 16 years
- Patients unable to attend practice premises
- Glucose tolerance tests
- Synacthen tests
- Any blood test requiring additional administration or medication
- Phlebotomy relating to the management of patients prescribed specialised medicines under a share care agreement with the acute provider (SMS LES)
- Phlebotomy where need is identified within General Practice for purpose of diagnosis or condition management within primary care (*PPO LES*)
- Phlebotomy that is otherwise explicitly commissioned

There is no requirement to:

- Record any additional information as this is a provision of phlebotomy only service.
- Operate any aspect of a recall system
- Review results or take any action associated with results as this is the responsibility of the requesting clinician from the Acute Provider.

3.5 Interdependence with other services/providers

The provider must ensure they work in partnership with local courier services for the collection and delivery of samples to the local laboratory safely and within allocated time frames.

4. Applicable Service Standards

4.1 Applicable national standards (eg NICE)

World Health Organisation (2010) WHO guidelines on drawing blood – best practice in phlebotomy

<https://www.who.int/publications/i/item/9789241599221>

5. Applicable quality requirements and CQUIN goals

5.1 Applicable Quality Requirements (See Schedule 4 Parts [A-D])

Quality requirement not identified at this time.

5.2 Applicable CQUIN goals (See Schedule 4 Part [E])

No CQUIN goal established or required

Each practice must ensure that any personnel involved in providing any aspect of care under this scheme has the necessary training, skill, and competency to do so.

Audit and Incident requirements are described below:

Formal incidents should continue to be reported to NHS England using the SEA form: - <https://www.google.com/url?sa=t&rct=j&q=&esrc=s&source=web&cd=&ved=2ahUKEwj3xI7O1cSSAxWjWUEAHdbhJSkQFnoECBcQAQ&url=https%3A%2F%2Fwww.england.nhs.uk%2Fsouth%2Fwp-content%2Fuploads%2Fsites%2F6%2F2021%2F08%2F2021-22-sea-incident-report-form.docx&usg=AOvVaw0yy1fjp4-ggQ32h1BCy6yL&opi=89978449>

For any other issues of poor quality of service, part of the system not working well or negative patient experience. Please continue to report via LfPSE (learning from patient safety events) You may have previously used PITCH to report incidents, however as LfPSE is being launched nationally, PITCH will no longer be required locally and has therefore been decommissioned.
NHS Devon can support with queries relating to LfPSE at d-icb.PSIRF@nhs.net.

6. Key Service Outcomes

Patients will receive a blood test in a primary care setting, reducing the number of patient contacts with secondary care and the number of journeys to hospital.

7. Financial Requirements

For 2026/27 NHS Devon has the budget to fund this service at 40.1p per weighted patient for the population of Devon and will pay this value to practices who sign up to provide the LES. Annual payments will be calculated on the list size at 1st January 2026 with the resultant value being paid in equal 12ths within the financial year of this specification.

Each provider will receive a fixed amount per month without the need to claim. There is a commitment by NHS Devon to monitor the number of blood tests completed. Practices will be required to submit their activity monthly via the Commissioning Intelligence portal using the following SNOMED code to identify the number of tests per month: -

- **Phlebotomy generated from secondary care done by practice**

XaK2I (165791000000107)

DDRB will be used as the start point for annual 'inflationary' review, with any deviation from DDRB uplift requiring clear rationale.

8. Location of Provider Premises

The Provider's (GP) Premises.